Institute of
Evidence-based
Policymaking

Evidence Analysis of the 2025 Ballot: Propositions LL & MM

Regarding the Healthy School Meals for All Program and Supplemental Nutritional Assistance Program

September 2025

About the Institute of Evidence-Based Policymaking

The Institute of Evidence-based Policymaking is committed to unbiased, evidence-based research adhering to the highest standards to inform elected executive decision makers. A huge delta exists between public policy decisions rooted in evidence and credible data, and public policy that is based on incomplete or biased data. But good data and best practices research also must be actionable and timely. By providing non-partisan, evidence-based research in the right time and manner, the Institute aims to equip decision-makers and the public with the best available information on public policy issues of importance to them, their families and communities.

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Executive Summary

Why this Report?

Ballot measures reach voters through the General Assembly, known as referred measures, or when citizens initiate measures. Leading up to election day, Colorado voters typically receive information from the Blue Book voting guide, which provides a nonpartisan summary of the measure, and from supporters and opponents of these ballot measures through paid media and direct voter contacts. Each side makes claims about the positive or negative consequences that will result if the ballot measure is passed.

Many of these ballot measures have significant consequences. But too often missing from the public discussion is an objective analysis of the evidence behind the supporters' and opponents' claims about the impact of the measures.

This report is designed to fill that gap by equipping voters with the best available unbiased information to make their decisions.

In keeping with established frameworks for characterizing levels of evidence, this report will identify evidence levels on a 1 through 5 scale, with 5 being the most rigorous, and evidence certainty as "high," "moderate," "low" or very low," so voters can put the information about ballot measures and the impact they may have in context.

The Ballot Measures

Two referred measures are on the ballot in November 2025, Propositions LL and MM.

Proposition LL would allow the state to keep the $12.4 million that was collected over the amount allowed by Proposition FF, the ballot measure in 2022 that passed to initiate the Healthy School Meals for All program.

If approved, the state would spend this additional revenue on the Healthy School Meals for All program instead of refunding the funds to taxpayers. This would impact households earning $300,000 and above annually who are taxed to fund the Healthy School Meals for All program.

Proposition MM would increase the amount of revenue collected for the Healthy School Meals for All program by reducing tax deductions for taxpayers earning $300,000 or more annually, effectively raising their taxes. The measure also expands the allowable uses for the revenue collected to include costs incurred by the state for implementing the Supplemental Nutrition Assistance Program (SNAP). Both of these measures require voter approval in accordance with the Taxpayer Bill of Rights (TABOR).

The Healthy School Meals for All program provides:

- state reimbursements to local schools that provide free meals, including breakfast and lunch, for all students at public schools that participate in the National School Lunch program and School Breakfast program;
- a grant program offering the greater of $5,000 or 25 cents per lunch to participating schools for a local food purchasing program;
- technical assistance grants for local food purchasing; and
- increased wages for school employees who prepare and serve lunch, the greater of $3,000 or 12 cents per school lunch.

The goal of the Healthy School Meals for All program is to provide school meals to all students at no cost.

What does the evidence say?

Analysis of the claims of positive impacts from the program:

Increased participation in school meals and nutritional quality
Evidence: High
Multiple studies suggest that offering free school meals increases participation in both breakfast and lunch, which improves nutrition particularly among schools that prioritize healthy meals.

Improves test scores and attendance
Evidence: Low
There is low evidence that suggests that test scores improve, and when improvements to test scores are observed, they are very minor. Attendance improvement is also low but may improve a couple years after universal free school meals has been implemented.

Reduces obesity
Evidence: Low

Research suggests minor improvements in body weight through participation in free school meals.

Improved nutrition decreases disciplinary action at school
Evidence: Moderate to Low
Evidence suggests that changing students' overall diet (rather than just adding supplements) has the biggest positive impact on negative behavior, especially for boys, showing a small effect on aggression, a medium effect on antisocial behavior, and a large effect on criminal offending.

Increased nutritional content of lunches prepared at school versus home
Evidence: Moderate
Despite parents' concerns about the quality of school meals, the study shows that school-prepared lunches are generally more nutritious than lunches brought from home.

Positive mental health impacts from reduced food insecurity
Evidence: Varied
Adequate nutrition can have a positive impact on the mental health of both students and parents:

- Depression: High. Food insecurity is strongly linked to depression across populations, with especially high risks among non-Hispanic Black and Hispanic families, children facing suicidality, and those experiencing more frequent or severe food insecurity, creating a cycle where depression and food insecurity reinforce one another.
- Anxiety: Low. Food insecurity shows a less consistent relationship with anxiety than with depression, appearing more clearly in adults than children and varying by race and ethnicity, with stronger evidence in non-Hispanic White populations than in Hispanic populations.
- Hyperactivity and Behavioral Issues: Moderate. Food insecurity, particularly in younger children and in cases of persistent or severe food insecurity, is associated with hyperactivity and behavioral issues.
- Stress: Low. Food insecurity may act as a chronic stressor tied to poor health outcomes, though evidence directly connecting food insecurity to psychological stress is mixed, possibly due to differences in how stress is measured and the unique ways parents experience food insecurity compared with the general population.

Analysis of the claims of negative impact from the program

Increased food waste
Evidence: Low
Longer lunch lines without increasing the allotted time to eat lunch results in students who opt into school lunches having less time to eat. One result is the potential for increased food waste. Results were inconclusive, however the study notes that average food waste is on par or below

Increase in state tax rates impact on economic activity
Evidence: Moderate
An increase in state income tax is found to reduce personal income growth and, in the long run, state income tax collections. There was no indication that the increase in state income taxes impacted unemployment or GDP.

The goal of the SNAP program is to ensure reliable access to food for individuals below a certain income level

What does the evidence say?

Analysis of the claims of positive impacts from the program:

Poverty reducing and non-nutritional effects of SNAP participation
Evidence: Varied
SNAP participation has benefits for parents and children beyond the food security and nutritional benefits it directly offers.

- Healthcare utilization: Moderate to high. SNAP participation has a positive effect on access to healthcare for both preventative and needed care.
- Family allocation of resources: Moderate. SNAP participation improves families' ability to manage monthly household expenses, including housing, utilities, and food costs.
- Impact on child development and behavior: Low. SNAP participation is tied to some improvements in child behavior and academic performance.
- Mental health: High. There is a strong link between SNAP participation and improved stress and depression for parents and improved mental health in children.
- Abuse and neglect: Moderate. Most studies associated participation in SNAP benefits with lower risk for child abuse and neglect.

Dietary sufficiency and quality for SNAP participants
Evidence: High

Research shows that participating in SNAP ensures that families have sufficient caloric intake, but the quality of the nutrition is lower for people on SNAP than for people not receiving SNAP benefits.

Analysis of the claims of negative impacts from the program

Barriers to SNAP Participation
Evidence: Moderate
There are various policy, administrative, environmental, access, and household barriers to SNAP participation that result in many households that are eligible for benefits not applying for or receiving benefits.

Why are we doing this analysis?

Ballot measures reach voters through two means. The General Assembly can refer a measure to the ballot, or the citizens can initiate measures. Ballot measures are currently characterized for the public by advocates and opponents through paid and earned media and other distribution channels, along with a descriptive summary and analysis from the Legislative Council Staff of the Colorado General Assembly. This analysis is called the Ballot Information Booklet, more commonly known as the “Blue Book.” The Blue Book voting guide provides an important resource for voters, but it does not include information about the efficacy of the components of the measures.

The objective of this report is to provide Colorado voters with clear, unbiased details about ballot measures earlier in the election cycle about the proposals they will consider in November. Too often missing is a thorough evidence-based analysis of each ballot measure. What is the level of evidence supporting the proposal? What is the likelihood that the proposal will achieve its intended objectives or impact? This analysis aims to fill that gap.

What is evidence-based policy?

Evidence-based policy, which has its roots in evidence-based medicine, has been defined as “the practice of informing public policy decisions through the use of scientific evidence and rigorous research…ensuring that policies are grounded in strong and credible empirical data, which can…enhance overall effectiveness.”[i]

The strength of research evidence is commonly assessed through classifying levels of evidence. Two of the most widely recognized frameworks for making that assessment in medicine which have more general application are the Oxford Centre for Evidence-Based Medicine (OCEBM) and the GRADE Working Group frameworks, as described in the Appendix in further detail.

Significant progress has been made in interpreting these medical research levels of evidence into the policymaking process. In keeping with these established frameworks for characterizing different levels of evidence, and for clarity and accessibility, this report will refer to evidence levels as defined below; and to evidence certainty as “high”, “moderate”, “low” or very low”. Additional information can be found in the Appendix.

On the Ballot in 2025: Propositions LL & MM

Two referred measures are on the ballot in November 2025; Propositions LL and MM. Below is a description of the two referred ballot measures. “Who” indicates the group that put the measure on the ballot; “what” indicates what the measure does; and “why” indicates why the measure is on the ballot.

Proposition LL

WHO

This is a referred measure from the Colorado General Assembly through House Bill 25-1274.

WHAT

Proposition LL allows the state to keep the $12.4 million that was collected over the amount allowed, plus interest, by Proposition FF, the ballot measure in 2022 that passed to initiate the Healthy School Meals for All program. If approved, the state would spend this additional revenue on the Healthy School Meals for All program instead of refunding the funds to taxpayers.

- Taxpayers under this proposition are households earning at least $300,000 annually who are taxed to fund the Healthy School Meals for All program.

WHY

The Colorado General Assembly is asking Colorado voters if the state can keep the revenue collected above the initial estimate. The Blue Book estimate for Proposition FF estimated there would be an increase of $100.7 million in additional taxes collected.[ii] Actual collections were $112.0 million, which is $11.3 million over the initial estimate.

- The Taxpayers’ Bill of Rights (TABOR) requires that if the revenue collected as a result of a voter-approved ballot measure that raises taxes exceeds the revenue estimate in the Blue Book guide, the excess revenue must be refunded to those who paid the tax.[iii]

Proposition MM

WHO

This is a referred measure from the Colorado General Assembly through House Bill 25-1274 and modified by Senate Bill 25B-003.

WHAT

Proposition MM increases the amount of revenue collected for the Healthy School Meals for All program by reducing tax deductions for taxpayers earning $300,000 or more annually, effectively raising their taxes. The measure also expands the allowable uses for the revenue collected to include costs incurred by the state for implementing the Supplemental Nutrition Assistance Program (SNAP).

WHY

The Taxpayers' Bill of Rights (TABOR) in the Colorado Constitution requires that any increase in taxes be approved by voters. [iv]

Background – Healthy School Meals for All

After an initial attempt to pass House Bill 22-087, the Colorado State Legislature passed House Bill 22-1414, sending a measure to the ballot to implement and fund the Healthy School Meals for All program. That ballot measure, Proposition FF, was approved by 56.7% of Colorado voters.[v]

The Healthy School Meals for All program began during the 2023-24 school year. The program includes the following components:

1. State reimbursements to local schools that provide free meals, including breakfast and lunch, for all students at public schools that participate in the National School Lunch program and School Breakfast program.
2. A grant program offering the greater of $5,000 or 25 cents per lunch to participating schools for a local food purchasing program.
3. Technical assistance grants for local food purchasing.
4. Increased wages for school employees who prepare and serve lunch, the greater of $3,000 or 12 cents per school lunch.

The goals of the program, as described in House Bill 22-1414[vi], were to:

- reduce hunger to boost health and wellbeing,
- improve effective learning and academic success,
- reduce stigma around free and reduced school meals, and
- support Colorado food systems (farmers and ranchers).

Prior to the implementation of the Healthy School Meals for All program, only students who were eligible for free and reduced school meals could receive them, and their families had to fill out paperwork to become eligible. Below are the income eligibility guidelines from the last school year, 2022-2023, prior to the implementation of Healthy School Meals for All.[vii]

Colorado Income Eligibility Guidelines for Free & Reduced Lunch School Year 2022-2023		
Household Size	Free Guidelines Yearly	Reduced-Price Guidelines Yearly
1	$17,667	$25,142
2	$23,803	$33,874
3	$29,939	$42,606
4	$36,075	$51,338
5	$42,211	$60,070
6	$48,347	$68,802
7	$54,483	$77,534
8	$60,619	$86,266
For each additional family member, add:	$6,136	$8,732
Source: Colorado Department of Education https://www.cde.state.co.us/communications/newsreleasejuly2022frl		

The measure also included a funding mechanism to pay for the program. Deductions for taxpayers earning $300,000 or more are reduced from their state income taxes than otherwise would be allowed. Starting in tax year 2023, single taxpayers earning $300,000 or more per year, the deduction is capped at $12,000, and for joint taxpayers, it is capped at $16,000. In 2020, the most recent year that data is published by the Colorado Department of Revenue, about 3.4% of all taxpayers had a Colorado taxable income of $300,000 or more per year, and their total Colorado taxable income made up 38.5% of all state taxable income that year. [viii]

It was estimated that state would collect $50.4 million in additional revenue during half of state fiscal year 2022-23 and $100.7 million in fiscal year 2023-24 for a total of $151.1 million for the first school year of the program [ix]. The reimbursement for free meals was estimated to cost up to $115.0 million, as well as another $758,000 in administrative costs. [x] Due to higher-than-expected demand for free school meals, the Healthy School Meals for All program cost $162.0 million during the first full school year. [xi] An additional $6.5 million was transferred from the State Education Fund to cover the difference between the revenue generated and the costs. [xii]

Due to the high demand for school meals and the shortage of funding, the Colorado General Assembly delayed the implementation of the grant programs included in Proposition FF from state fiscal year 2024-25 until 2025-26.[xiii] The grant programs delayed are $5 million for the local school food purchasing program, $5 million for a technical assistance grant program, and $8.4 million for additional school food worker wages.

Senate Bill 25B-003 further modified the allocation of revenue to the various programs under the Healthy School Meals for All program, which will depend on how much money the state has to put in a reserve fund to ensure the school meal program can be fully funded.

Senate Bill 25B-003 Changes to Healthy School Meals for All Revenue Allocations				
Reserve Percentage	**Technical Assistance Grants**	**School Food Worker Wage Stipends**	**Local Food Purchasing Grants**	**SNAP Allocations**
Less than 10%	$250,000	6¢ per lunch	Remaining amount	None
10% to 25%	$2,500,000	6¢ per lunch	10¢ - 12.5¢ per lunch	None
25% to 35%	$3,750,000	9¢ per lunch	16¢ - 18.75¢ per lunch	None
35% or more	$5,000,000	12¢ per lunch	25¢ per lunch	Any remaining funds up to the amount needed for SNAP administration
Source: Senate Bill 25B-003 Fiscal Note.				

Background – SNAP Changes Under H.R. 1

SNAP, or Supplemental Nutrition Assistance Program (previously referred to as "food stamps"), provides families and individuals with a monthly subsidy to help them purchase food. Eligibility is determined by household size and income, which must be under 200% of the Federal Poverty Level to qualify, among other requirements. Monthly allotments are shown below. The funds can be spent on food for home preparation, but they are not allowed for purchases of household supplies, personal hygiene, alcoholic beverages, and others.

SNAP Income Limits and Allotments by Household Size		
Household Size	Gross Monthly Income Limit	Maximum Monthly Allotments
1	$2,510	$292
2	$3,408	$536
3	$4,304	$768
4	$5,200	$975
5	$6,098	$1,158
6	$6,994	$1,390
7	$7,890	$1,536
8	$8,788	$1,756
Additional Member	+$898	+$23
Source: Colorado Department of Human Services https://cdhs.colorado.gov/snap		

In 2024, an average of 594,526 Coloradans were receiving SNAP benefits, which totaled 10.0% of the state's population.[xiv] The average monthly benefit amount per person was $190. This is an increase from 7.8% of the population receiving benefits in 2019 prior to the COVID-19 pandemic.[xv]

SNAP is administered at the federal level by the U.S. Department of Agriculture, and SNAP payments have been funded exclusively by the federal government. In Colorado, SNAP benefits are administered by the state and counties. The state transfers funds to each county based on their client counts, and those administrative costs have been shared between the state and federal governments. Baseline eligibility is set by the federal government and can be expanded by states using the Broad-Based Categorial Eligibility provision.[xvi]

A few changes made under federal bill H.R. 1 (One Big Beautiful Bill Act)[xvii] impact SNAP and how it is funded:

1. Starting in October 2027, states will be required to fund a portion of SNAP benefits if their payment error rate is over 6.0%. The cost sharing percentages are:

Payment Error Rate (PER)	Federal Government SNAP Funding
<6%	100%
6-8%	95%
8-10%	90%
>10%	85%
PER*1.5 = >20%	Implementation of cost share delayed

According to the U.S. Department of Agriculture (USDA), SNAP payment error rates: "measure the accuracy of each state's eligibility and benefit determinations while the national performance measure or national payment error rate represents the average of these rates, weighted by state caseload sizes. Payment errors include both underpayments and overpayments."[xviii]

2. Federal reimbursement rates to states for SNAP administration will decrease from 50% to 25% beginning in October 2026.

3. Work requirements changed:
    a. General work requirements apply to individuals between the ages of 17 and 64, instead of 16- to 59-year-olds.
    b. The Able-Bodied Adults Without Dependents provision changed as well, increasing the work requirement and time limit (three months of benefits within a three-year period) and defining dependents as children under age 14. The exemption for homeless individuals, veterans, and individuals 24 years old and younger who aged out of foster care has been removed.

Colorado's SNAP payment error rate in fiscal year 2024 was 9.97%, compared to the U.S. average of 10.93%.[xix] Payment error rates occur when state and local officials process applications or when SNAP applicants enter their information incorrectly. Eight states were under the 6.0% payment error rate threshold in 2024.[xx] Approximately $1.36 billion was allocated to Colorado SNAP recipients during 2024. With the state's error rate of just under 10%, the state would be required to pay an additional $136 million under the cost share provision starting in federal fiscal year 2028. The state's cost share will be determined by the payment error rate during fiscal years 2025 and 2026 and the number of SNAP beneficiaries when the cost share begins.

Additional changes to the administration of SNAP include what utilities can count as part of a SNAP recipient's utility costs. These administrative changes may not have a direct cost, but they increase the amount of time state and county governments must spend on administering the program.

What does the evidence say?

The goal of the Healthy School Meals for All program is to provide school meals to all students at no cost. The positive impacts of providing meals for all students include:

- providing nutrition to all students despite their ability to pay and their willingness to participate in the program, which requires public acknowledgement that a family is low income,
- potentially improved school-related outcomes, like test scores, attendance, and disciplinary actions, and
- reducing the time and money spent by families on preparing school meals.

The potentially negative impacts of the program include:

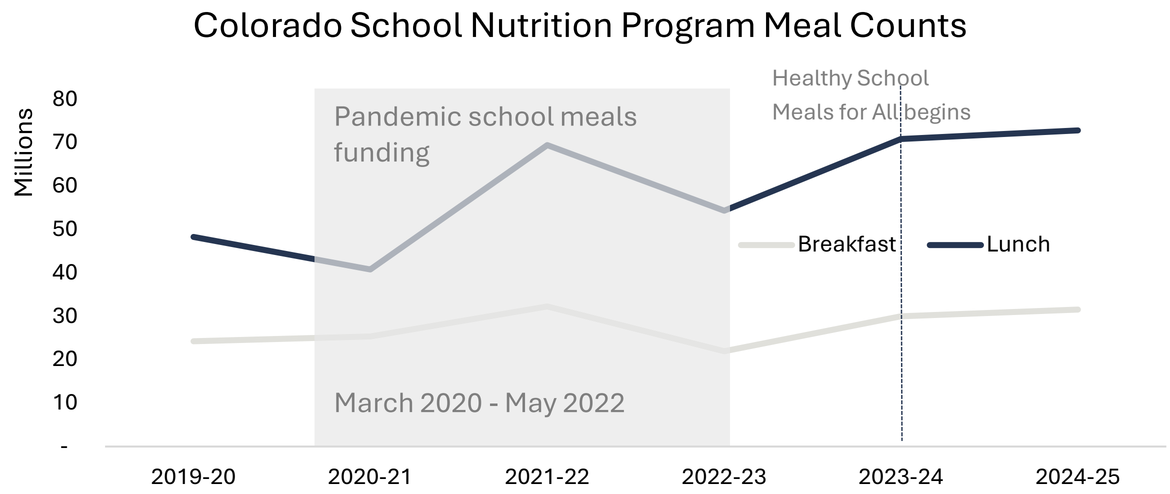
- increased taxes on families earning $300,000 or more per year to pay for meals for all students, many of whom can afford to pay for meals, and
- less time for students to eat due to long lunch lines which may result in increased food waste.

The goal of the SNAP program is to ensure reliable access to food for individuals below a certain income level, which is 200% of the federal poverty line in Colorado.

Healthy School Meals for All: First Two Years

The program started in school year 2023-24 and all 183 school nutrition programs in Colorado opted into the program. School nutrition programs served 37% more lunches and 30% more breakfasts during the first year of implementation over the prior school year. This represents an annual increase of 16.5 million lunches and 8.0 million breakfasts. Preliminary data for school year 2024-25 indicates a more muted increase of about 1.5 million more breakfasts served and about 2.0 million additional lunches, which represent a 5.3% and 2.8% increase, respectively.[xxi] School year 2022-23 experienced a decline in school meal participation after COVID-19 era stimulus funding for free school meals ended. The data for school year 2024-25 has not been published yet. According to a report by the Urban Institute, states that have implemented universal free school meals saw a 1% to 6%

increase in participation between the first and second years, which is in line with the change in Colorado.[xxii]



Source: Colorado Department of Education.

Additional research presented findings of a survey of Colorado school food service administrators.[xxiii] The survey was regarding the transition away from free school meals for all during the COVID-19 pandemic, which began in March 2020 and ended at the end of school year 2021-22, to requiring that students pay for meals again in school year 2022-23. The survey reported the most common challenges from that transition:

- 62% increase in unpaid meal charges/debt
- 62% reduced school meal participation
- 55% increased food service staffing challenges
- 50% reduced foodservice revenues
- 47% increased ease of obtaining income information from families
- 42% increased paperwork and administrative burden

These challenges were exacerbated by a lack of funding for school lunches. About 75% of food service directors reported that current reimbursements from the federal government did not cover the full cost of producing school breakfasts and lunches. They also report needing additional funding to serve fresh, local produce.

Additionally, the most common challenges with cafeteria operations in SY 2022-23 included:

- 63% not enough time for students to eat

- 55% not enough refrigeration/freezer space
- 40% long time in line for students to get meals
- 37% not enough kitchen preparation space
- 32% inadequate meal service space
- 26% inadequate/crowded dining space

Evidence from Research Literature

Dozens of studies have been done on the outcomes associated with free school meal programs. Most of these studies are done in school districts or systems that have implemented Community Eligibility Provision programs, in which the federal government provides universal free school meals for districts with high percentages of low-income families,[xxiv] since state-funded free school meal programs were first implemented in school year 2022-23. Because there are so many different studies with various populations, outcomes measured, and quality, this evidence analysis focuses on systematic reviews and meta analyses. Systematic reviews combine multiple studies, evaluate the quality of the studies, aggregate their data, and analyze the results. Meta analyses do the same but typically focus on including randomized controlled trials. Both systematic reviews and meta analyses are Level 4 – Evidence Informed in the levels of evidence scale.

The Levels of Evidence indicate the type and quality of research done, while the Evidence Strength indicates the reliability of the findings. Additional information about the evidence rankings can be found in the Appendix.

Levels.of.Evidence

- Step 5: Proven
- Step 4: Evidence Informed
- Steps 1-3: Theory Informed
- No Step: Opinion-based

Evidence.Certainty

- High
- Moderate
- Low
- Very Low

<table>
<tr><th></th><th>Outcome of Universal School Meal Programs</th><th>Evidence Level</th><th>Evidence Certainty</th></tr>
<tr><td rowspan="15">Universal Free School Meals</td><td>Meal Participation – Lunch</td><td rowspan="23">4 – Evidence informed</td><td>Moderate</td></tr>
<tr><td>Meal Participation – Breakfast</td><td>Very low</td></tr>
<tr><td>Improved Attendance</td><td>Low</td></tr>
<tr><td>Lower Obesity</td><td>Very low to Moderate</td></tr>
<tr><td>Less Food Insecurity</td><td>Low</td></tr>
<tr><td>Improved Academic Performance</td><td>Low</td></tr>
<tr><td>Less Disciplinary Action</td><td>Low</td></tr>
<tr><td>Higher Nutritional Content of Meals</td><td>Moderate</td></tr>
<tr><td>Mental Health Impacts</td><td>Moderate</td></tr>
<tr><td align="right">Depression</td><td>High</td></tr>
<tr><td align="right">Anxiety</td><td>Low</td></tr>
<tr><td align="right">Hyperactivity and Behavioral Issues</td><td>Moderate</td></tr>
<tr><td align="right">Stress</td><td>Low</td></tr>
<tr><td>Improved School Finances</td><td>Low</td></tr>
<tr><td>Increased Food Waste</td><td>Low</td></tr>
<tr><td rowspan="8">SNAP Participation</td><td>Higher Healthcare Utilization</td><td>Moderate to high</td></tr>
<tr><td>Improved Household Finances</td><td>Moderate</td></tr>
<tr><td>Improved Child Development and School Performance</td><td>Low</td></tr>
<tr><td>Mental Health</td><td>High</td></tr>
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<tr><td>Barriers to SNAP Participation</td><td>Moderate</td></tr>
<tr><td>Taxation</td><td>Increase in State Tax Rates Impact on Economic Activity</td><td>3 – Theory informed</td><td>Moderate to low</td></tr>
</table>

School Meals Potential Positive Outcomes

Student Participation, Attendance, Academic Performance, Diet Quality, Food Security and Body Mass Index

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Varies based on outcome

A systematic review of 47 studies was done to determine the relationship between universal free school meals and a variety of outcome metrics from nutritional to student academic performance.[xxv]

- **School Meal Participation**: There is **strong** evidence to show that school meal participation increased when universal free school meals are offered. The various studies included in the review examined the percent increases in participation, which ranged from about 6% increase in lunch participation to 36% and from about 4% participation in breakfast to 21%. Notably, studies that broke down participation rates into socioeconomic and demographic groups showed that minority students were more likely to participate in free school meals, and a study in Community Eligibility Provision (CEP) schools showed participation rates higher among students not previously eligible for free or reduced price meals and slightly lower participation among low-income students in CEP schools compared to non-CEP schools.
- **Diet Quality**: There is **moderate** evidence quality among studies evaluating diet quality, particularly in schools with high nutritional standards as school food service staff could spend additional time improving school meal quality rather than processing free and reduced school meal paperwork.
- **Food Insecurity**: Few studies evaluated universal free school meals impact on food insecurity, but the preliminary evidence suggests there was improved food security among students and families suggesting **low** evidence certainty.
- **Attendance**: There is **low** evidence quality due to few studies on the impacts of free school meals on attendance, but preliminary research suggests that there is a lag of about two years to observe improved attendance after implementation of universal school meal programs.
- **Academic Performance**: There was **low** evidence that test scores improved when students' nutrition improved through having access to free school meals. Depending on the study, there was either no change or a very small

increase observed in test scores primarily among elementary and middle school students.

- **Body Mass Index**: Evidence suggests with **moderate** certainty that universal free school meals programs do not increase the prevalence of obesity, with some finding a reduction in obesity.
- **School Finances**: Few studies have been conducted on the potential benefits to school finances; however, evidence is **low** that schools benefit financially. Most schools likely to participate in Community Eligibility Provision programs already have a high percentage of low-income students that increase their reimbursement rates.

Meal Participation, Attendance, and Obesity

- ➢ Evidence Level: 4 – Evidence informed program or practice
- ➢ Evidence Certainty – Moderate to Very Low

A systematic review examined studies on universal free school meal (UFSM) programs, such as the Community Eligibility Provision (CEP), and their impact on students in U.S. schools beginning in August 2012 and excluding the years impacted by the COVID-19 pandemic. Study outcomes included meal participation rates, attendance, dietary intake and quality, food waste, economic impacts, food insecurity, anthropometrics, disciplinary actions, stigma, and shaming. Across more than 11,000 schools:

- There was **moderate** certainty in higher participation in lunch in CEP programs.
- There was **very low** certainty in higher breakfast participation rates.
- There was **low** certainty in that attendance was either unchanged or slightly improved.
- Single studies with **very low** certainty found lower obesity rates and fewer suspensions in CEP schools.

Outcomes like diet quality, food security, and stigma were not well studied, and most findings were limited by small sample sizes or indirect data. Overall, universal free school meal programs appear to increase meal participation and may provide modest health and behavioral benefits, but stronger, broader research is needed to confirm their full impact. [xxvi]

Improved Nutrition Decreases Disciplinary Action at School

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Low

The strongest research available is a systematic review that evaluated 50 studies conducted between 1978 and 2023 on the impacts of nutrition on reducing aggression, antisocial, or criminal behavior on youth up to the age of 24. Additional nutrition was provided through dietary changes, the addition of nutrients to foods (fortification), or taking supplements. The strongest evidence suggests that changing overall diet (rather than just adding supplements) has the biggest positive impact, especially for boys, though results vary a lot between studies. Broad diet changes showed a small effect on aggression, a medium effect on antisocial behavior, and a large effect on criminal offending. Omega-3 supplements had little to no effect on aggression or antisocial behavior, while vitamin D showed a small-to-moderate effect on antisocial behavior. Because most of the research is focused on younger children with conditions like ADHD, and many studies are small or have a high risk of bias, the findings should be seen as early evidence rather than firm conclusions. Overall, nutrition does seem to play a role in behavior, but more high-quality studies are needed to understand when and how it works best. [xxvii]

Nutritional Content of Lunches Prepared at School versus Home

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Moderate

This systematic review and meta-analysis analyzed studies on lunches brought from home (LBFH) compared with school lunches provided through the National School Lunch Program (NSLP). About 40% of U.S. children bring meals from home, yet these lunches are typically less nutritious, with 50% including fruit, 17% contain vegetables, and 25% dairy. Additionally, many feature snacks, sweets, or sugary drinks. Compared with NSLP meals, LBFH had less calcium, protein, iron, fiber, and vitamin A, and more carbohydrates and saturated fat. Intervention programs aimed at improving the quality of LBFH showed no impact. LBFH cost slightly less than school meals on average, $1.81 vs. $1.98, though when accounting for preparation time, homemade meals for younger children were four times more expensive. Parents often preferred sending LBFH due to concerns about school meal quality, the amount of time students have to eat when they bring their lunch, and whether their children would eat school lunch. Overall, the evidence shows that LBFH are generally less healthy than NSLP meals, suggesting that efforts to improve perceptions of and participation in school meal programs—especially as more states offer free meals to all students—are important for ensuring children receive balanced and affordable nutrition.[xxviii]

Mental Health Impacts of Food Insecurity

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Varies Based on Condition (see below)

This systematic review evaluated 108 studies published between 1990 and 2020 that assessed the link between food insecurity and mental health in children and parents.[xxix] The studies looked at the impact of adequate nutrition on depression, anxiety, externalizing behavior, internalizing behavior, hyperactivity, and stress, with the following results:

- **Depression**:
  Evidence Strength: High

  Food insecurity is strongly linked to depression across populations, with especially high risks among non-Hispanic Black and Hispanic families, children facing suicidality, and those experiencing more frequent or severe food insecurity, creating a cycle where depression and food insecurity reinforce one another.
- **Anxiety**:
  Evidence Strength: Low

  Food insecurity shows a less consistent relationship with anxiety than with depression, appearing more clearly in adults than children and varying by race and ethnicity, with stronger evidence in non-Hispanic White populations than in Hispanic populations.

- **Hyperactivity and Behavioral Issues**:
  Evidence Strength: Moderate

  Food insecurity, particularly in younger children and in cases of persistent or severe food insecurity, is associated with hyperactivity and behavioral issues.
- **Stress**:
  Evidence Strength: Low

  Food insecurity may act as a chronic stressor tied to poor health outcomes, though evidence directly connecting food insecurity to psychological stress is mixed, possibly due to differences in how stress is measured and the unique ways parents experience food insecurity compared with the general population.

School Meals Potential Negative Outcomes

Increased Food Waste:

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Low

A systematic review of 53 studies on food waste in the National School Lunch Program from 1978 to 2015 found that the methods used to measure waste varied too widely to draw any strong conclusions, and that methods need to be standardized moving forward. Most studies estimated food waste at over 30% and no studies reported under 5% food waste.[xxx] The average food waste in the U.S. is estimated at 30% to 40%.

SNAP Research

Poverty-reducing and Non-nutritional Effects of SNAP Participation

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Varied

This scoping review of the non-nutritional impacts of families receiving SNAP benefits included 40 articles to review the existing research on poverty reduction and various aspects of familial stress:[xxxi]

- **Healthcare Utilization for Children and Parents**: Most studies show that SNAP participation increases children's and parents' access to healthcare, particularly for preventive care and needed services. While a few mixed findings exist, the overall evidence is **moderate to high**, suggesting a reliable positive association.
- **Familial Allocation of Resources**: SNAP helps families better manage housing, utility, and food-related expenses, though benefits often run out before the month's end. Evidence is **moderate**, with findings of improved short-term resource allocation but ongoing long-term challenges.
- **Impact on Child Development and Behavior**: SNAP is linked to some improvements in child behavior, academic performance, and long-term outcomes, but results are mixed with several studies showing negative effects tied to timing of benefits. The evidence is **low**, pointing to promising but uncertain impacts that require further research.
- **Mental Health**: Across most studies, SNAP participation is strongly associated with reduced parental stress and depression and improved mental health in children. The evidence is **high**, showing a consistent pattern of positive effects on family mental health.
- **Abuse or Neglect**: Most studies find that SNAP participation reduces child abuse and neglect, though some groups (e.g., parents with disabilities) may experience higher risks. The evidence is **moderate**, supporting SNAP's role in lowering family stressors that contribute to maltreatment.

Dietary Sufficiency and Quality for SNAP Participants

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: High

This systematic review evaluated 25 studies completed between 2003 and 2014 to understand the quality of caloric intake by SNAP participants compared to higher income groups that do not receive SNAP benefits. The evidence is strong that while SNAP participants generally consume enough calories, their overall diet quality, especially among adults, is lower than that of higher-income nonparticipants. Adults showed significantly lower Healthy Eating Index scores, and children had poorer diets compared to higher-income peers. Federal school meal programs appear to buffer this effect for children, helping maintain adequate calorie intake and improving diet quality.[xxxii]

Barriers to SNAP Participation

- Evidence Level: 4 – Evidence informed program or practice
- Evidence Certainty: Moderate

A systematic review of 17 studies was done to determine the primary barriers to SNAP participation at the administrative, environmental, community, policy, and household levels:[xxxiii]

Policy & Administrative Barriers have the highest impact on participation through:

- Strict eligibility rules like immigration restrictions, felony drug bans, and asset tests that exclude many income-eligible households.
- Recertification processes cause families to cycle in and out of participation, leading to loss of benefits and gaps in food access.
- Administrative burdens like cumbersome applications, verification, paperwork, staff capacity that prevent eligible households from enrolling or staying enrolled.

Environmental & Access Barriers have a moderate impact, since households located in urban areas likely have more access than rural households, through:

- Distance to enrollment offices or authorized retailers makes it harder to access for rural families and those without transportation.
- Limited online redemption and benefit access were especially challenging during COVID-19, reducing uptake.

Household-Level Barriers have a moderate impact on households through:

- Stigma, embarrassment, or fear of deportation discourage participation even when families are eligible.
- Competing priorities like managing work, childcare, or health conditions lead to delays or nonparticipation.

- Perception of low benefit value results in some families feeling the effort of applying or recertifying is not worth the benefits, which run out before the end of the month for many families.

Funding Research

Increase in State Tax Rates Impact on Economic Activity

- Evidence Level: 3 – Theory informed program or practice
- Evidence Certainty: Moderate to Low

An economic analysis looks at the impact of three different state tax rates impact state gross domestic product (GDP) growth, personal income, the labor market, and state government revenue. The three taxes evaluated were corporate income tax, individual income tax, and sales tax. Research indicates that state individual income tax will have a negative relationship with GDP and personal income growth. Raising the individual income tax rate by 1.0% would lower personal income growth by an estimated 0.93%. There was a similar impact on GDP growth; however, the confidence level in that impact is lower as the results were not statistically significant. The findings also show no impact of increased individual income tax on unemployment rates and a significant increase in state tax revenue as a percentage of GDP. However, the study also indicates that raising income tax rates permanently would result in lower total tax collection due to its negative impact on GDP and personal income growth. In summary, an increase in individual income tax results in lower economic growth and contemporaneous state tax revenue collections. [xxxiv]

State Budget and Economic Impact

Revenue

Proposition MM raises taxes on taxpayers who earn at least $300,000 annually by lowering the amount they can deduct on their state tax return from $16,000 to $2,000 for joint filers and from $12,000 to $1,000 for single filers. This would increase their taxable income by $14,000 for joint filers and $11,000 for single filers, increasing their average state taxes by

$616 or $484, respectively. Other groups will not see any additional taxes under these ballot measures.

The total amount of new tax revenue collected is limited to $95 million annually, since that number is included in the question on the ballot and as such is limited by the Taxpayer's Bill of Rights (TABOR). The measure is currently estimated to generate $50.7 million during the first six months of 2026 (half of state fiscal year 2025-26) and $103.0 million during the second half of 2026 and the first half of 2027, which is state fiscal year 2026-27. Any revenue collected up to $95 million is not subject to the provisions under TABOR, since it is a voter approved revenue increase. Revenue in excess of the $95 million limit will be refunded to taxpayers unless another measure is referred to the ballot for voter approval to keep the additional revenue. The revenue collected under Proposition MM is in addition to the revenue collected under Proposition FF.

Due to changes in the federal tax code under H.R. 1, there is elevated risk in tax revenue estimates currently. If the revenue estimates are below the estimates provided, there may be insufficient revenue to fund the Healthy School Meals for All program, the related grant programs, and SNAP administrative costs. Because Colorado's state income taxes are tied to federal taxable income, any additional changes made to the federal tax code will impact Colorado's income tax collections.[xxxv]

Expenditures

The additional revenue generated for the program will be allocated starting in state fiscal year 2026-27 to the following programs in this order:

1. Healthy School Meals for All fully funded including a 35% reserve,
2. grants for local food purchasing and technical assistance and for wages for school food service workers, and
3. SNAP administration.

State Expenditures under Proposition MM[xxxvi]			
	FY25-26	FY26-27	FY27-28
Total Expenditures	$145.0 million	$215.2 million	$257.7 million
HSMA	$145.0 million	$155.0 million	$155.0 million
Food Purchasing Grants and Food Service Worker Pay	NA	$33 million	$33 million
SNAP Administration	NA	$27.2 million	up to $69.7 million

SNAP Costs

SNAP administration cost sharing has changed under H.R. 1. Instead of the federal government reimbursing states for 50% of the administrative costs, it will now reimburse only 25%. That change equates to an estimated $27.2 million in additional costs during the state fiscal year 2026-27 and $69.7 million in state fiscal year 2027-28. The state will be responsible for covering these costs out of the state budget whether Proposition MM passes or not. If Proposition MM does not pass, the state will have to find another way to increase revenue or cut spending from other programs across the state budget to pay for SNAP administration.

What happens if Proposition LL and MM do not pass?

There are four different scenarios possible depending on whether Propositions LL and MM pass or fail. The outcomes of these scenarios on the Healthy School Meals for All program are as follows:

Propositions LL and MM Pass

- The state will keep the $12.4 million and allocate it to the Healthy School Meals for All program.
- State income tax deductions will decrease for single or joint filers earning $300,000 or above in 2026, which will generate up to $95 million in state revenue annually.
- The increased state revenue will pay for the ongoing implementation of the Healthy School Meals for All program at all schools, the funding of the grant programs, and at least partial funding of SNAP administrative costs.

Proposition LL Passes but Proposition MM Fails

- The state will keep the $12.4 million and allocate it to the Healthy School Meals for All program.
- There will be no change to tax deductions for single or joint filers earning $300,000 or above, but state revenue will increase by an estimated $13.8 million annually due to changes from H.R. 1.

- Free school meals will be available during the current school year, but future free school meals will depend on funding levels and demand.
- The state will be required to spend at least $1 million on local food purchasing and technical assistance grant programs.
- SNAP administrative costs will not be covered by this funding and will have to be funded out of existing state funds or the state will be required to increase revenue separate from this program.

Proposition LL Fails but Proposition MM Passes

- The state will be required to refund $12.4 million to households earning at least $300,000 annually.
- State income tax deductions will decrease for single or joint filers earning $300,000 or above in 2026, which will generate up to $95 million in state revenue annually.
- The increased state revenue will pay for the ongoing implementation of the Healthy School Meals for All program at all schools, the funding of the grant programs, and at least partial funding of SNAP administrative costs.

Propositions LL and MM Fail

- The state will be required to refund $12.4 million to households earning at least $300,000 annually.
- Deduction limits on taxpayers earning at least $300,000 will be increased over what was initially passed in Proposition FF such that no additional state revenue is collected over and above the limit set in that measure in 2022.
- The Healthy School Meals for All program will be limited to schools eligible based on the percentage of students that qualify as low-income under federal school meals programs.
- The grant programs under the Healthy School Meals for All programs will not receive funding.
- SNAP administrative costs will not be covered by this funding and will have to be funded out of existing state funds or the state will be required to increase revenue separate from this program.

Appendix: Levels of Evidence

The strength of research evidence is commonly assessed through different levels of evidence. Two of the most widely recognized frameworks for making that assessment in medicine which have more general application are the Oxford Centre for Evidence-Based Medicine (OCEBM) and the GRADE Working Group frameworks.

The OCEBM Levels of Evidence (2011) classify evidence on a continuum from Level 1, which includes systematic reviews and high-quality randomized controlled trials (RCTs), through Level 2 (individual RCTs or cohort studies), Level 3 (case-control studies), Level 4 (case series and low-quality cohort or case-control studies), and Level 5 (expert opinion without explicit critical appraisal).

In parallel, the GRADE system evaluates both the quality of evidence and the strength of recommendations, categorizing evidence as high?moderate?low?or very low. Under GRADE, well-conducted RCTs typically start as high-quality evidence, while observational studies begin as low-quality but may be upgraded or downgraded based on factors such as risk of bias, consistency, and directness of results. Together, these frameworks provide a rigorous and transparent approach for weighing research findings and guiding practice and policy decisions.

Significant progress has been made in interpreting these medical research levels of evidence into the policymaking process. Organizations like Results for America advise governments on establishing definitions and best practices in their organizations or government entities in order to put the research and evidence into practice in policymaking.

In keeping with these established frameworks for characterizing different levels of evidence, and for clarity and accessibility, this report will refer to evidence levels per the definitions below and to evidence certainty as “high”, “moderate”, “low” or very low” as defined below.[xxxvii]

Level 5 - Proven: A program or practice that reflects a high or well-supported level of confidence of effectiveness, ineffectiveness, or harmfulness as determined by one or more high-quality randomized control trials, multiple evaluations with strong comparison groups, or an equivalent measure. Types of research required to reach Step 5 include randomized control trials (RCTs), systematic reviews, and meta-analyses, among others.

Level 4 - Evidence-informed: A program or practice that reflects a moderate, supported, or promising level of confidence of effectiveness, ineffectiveness, or harmfulness as determined by an evaluation with a comparison group, multiple pre- and post-evaluations, or an equivalent measure.

Level 3 through 1 - Theory-informed: A program or practice that reflects a moderate to low or promising level of confidence of effectiveness, ineffectiveness, or harmfulness as determined by tracking and evaluating performance measures including pre- and post-intervention evaluation of program outcomes, evaluation of program outputs, identification and implementation of a theory of change, or equivalent measures.

No Level - Opinion-based: A program or practice that reflects a low level of confidence of effectiveness, ineffectiveness, or harmfulness, as based on satisfaction surveys, personal experience, or for which there is no existing evidence about the effectiveness, ineffectiveness, or harmfulness of the program or practice.

Evidence Certainty rankings are gleaned from the author's interpretation of their results. There are various methods to make these determinations, including through the GRADE system and different quantitative techniques.

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